

FOR QUALIFIED NURSES IN INDIA

The Indian Nurse's Guide to Working in the UK

Your step-by-step roadmap from India to starting your nursing career in the UK.

Understand the registration process, English requirements, assessments, job search, sponsorship, relocation and what to expect when you start working in the UK.

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Northward Care publishes general information about NMC registration and UK immigration rules. This guide is not immigration advice and must not be relied on as advice about your own circumstances.

What's inside

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Read this first

Registration rules are set by the Nursing and Midwifery Council. Immigration rules are set by the UK Government. Both change, sometimes at short notice, and both organisations publish their own current requirements. Everything in this guide was checked against official sources in September 2026 — but before you act on any requirement, fee or score in these pages, confirm it on the official website. The Official sources page at the end lists every link you need.

The big picture: your UK nursing journey

Most nurses who get stuck do not get stuck because the process is hard. They get stuck because nobody showed them the whole map before they started. Here it is.

PHASE ONE · GETTING YOURSELF READY

1 Understand the UK system

Who regulates nursing, who employs nurses, and who decides immigration.

2 Check your eligibility

Whether your qualification and registration history allow you to apply.

3 Prepare your documents

Start collecting early. Documents are a common source of avoidable delay.

4 Meet the English language requirements

Either an accepted test, or one of the alternative forms of evidence.

PHASE TWO · PROFESSIONAL REGISTRATION

5 Apply for NMC registration

Create your account and submit your eligibility and qualification application.

6 Complete the Test of Competence

The CBT (Computer Based Test), which you can sit in India, and the OSCE (Objective Structured Clinical Examination), which is sat in the UK — before or after you move.

7 Complete your NMC registration

Health, character, English and indemnity evidence, then the NMC's decision.

PHASE THREE · FINDING YOUR ROLE

8 Search and apply for suitable roles

Across NHS, independent healthcare, social care and community employers.

9 Receive a job offer

From a specific employer, for a specific role, in a specific place of work.

10 Understand sponsorship

Whether the employer holds a sponsor licence and whether the role is sponsored.

PHASE FOUR · MOVING AND STARTING

11 Complete your visa process

Your application, your evidence, your biometrics, the decision.

12 Prepare for relocation

Documents, accommodation, travel, money, phone, contacts.

13 Arrive in the UK

Often followed quickly by your OSCE, if you have not already sat it.

14 Complete employer onboarding

Induction, identity checks, occupational health, mandatory training.

15 Start your UK nursing role

With support, supervision and time to learn local systems.

WHAT THIS ACTUALLY MEANS

These stages overlap. Many nurses are collecting documents while revising for the CBT, and talking to employers while their NMC application is still open. Others sit the OSCE only after arriving in the UK with a job already secured. There is no single correct sequence and no standard timeline — journeys commonly run from several months to well over a year, depending on your own circumstances, how quickly evidence arrives, and how fast each organisation processes its part.

Three different bodies. Three different jobs.

More confusion comes from this one point than from anything else in the process. Once you separate these three, most of the contradictory advice you will read online sorts itself out.

REGULATOR

The NMC

The Nursing and Midwifery Council. It decides whether you may practise as a registered nurse in the UK, assesses your qualification, sets the English language standard for registration, runs the Test of Competence and issues your PIN. It does not find you a job and has no role in immigration.

EMPLOYER

The employer

An NHS organisation, an independent hospital, a care home provider, a hospice, a community service. It decides whether to offer you a role, what that role involves, what it pays, and — if it holds a sponsor licence — whether to sponsor you. It does not grant visas and does not register you.

GOVERNMENT

The Home Office

Part of the UK Government. It licenses employers to sponsor workers, sets the immigration rules and salary and English requirements for visas, and decides your visa application. It does not assess your nursing qualification and does not employ you.

Why the distinction matters so much

Each body has its own requirements, its own evidence, its own timescales and its own way of telling you the answer. They do not talk to each other on your behalf, and satisfying one does not satisfy another.

The four statements that catch people out

"I'm NMC registered, so I can move"	Registration is permission to practise , not permission to enter the UK . You still need a job and, in most cases, a visa.
"I have a job offer, so my visa is fine"	An offer is a decision by an employer. A visa is a decision by the Home Office, assessed against its own rules and evidence.
"I passed IELTS, so my English is done"	The NMC and the Home Office each set their own English requirement. They are not automatically the same thing — see chapter 07.
"My agent is handling it"	Your NMC application and your visa application are yours. Whoever supports you, you are the one accountable for the evidence in them.

A useful habit from day one

Every time you read a requirement — anywhere, including in this guide — ask yourself: **which of the three set this?** Then check it on that organisation's own website. It takes thirty seconds and it is a habit worth building early.

How nursing is organised in the UK

Registered nurses in the UK work in a wide range of places. Understanding the landscape early helps you read job adverts properly and decide what kind of role you actually want.

The NHS

The National Health Service is the UK's publicly funded healthcare system, free at the point of use for most services. It is the largest single employer of registered nurses in the country. NHS nursing roles sit in hospitals, in community services, in mental health services, in ambulance services and in primary care. NHS organisations are also subject to national pay and terms frameworks, which is where terms like "Band 5" come from — see chapter 04.

The NHS is not one employer. In England it is made up of many separate NHS trusts and foundation trusts, each recruiting in its own right, with integrated care boards planning services across an area. Scotland, Wales and Northern Ireland are organised differently again. You apply to an organisation, not to "the NHS".

Private and independent healthcare

Independent hospitals and healthcare providers also employ registered nurses across surgical, medical, diagnostic, oncology, cosmetic and specialist services. Some work closely with the NHS and deliver NHS-funded care; others are wholly private. Pay, terms, shift patterns and career structures are set by each employer and are not bound by NHS national frameworks.

Nursing homes and care homes with nursing

Registered nurses are central to adult social care. In a care home with nursing, the registered nurse is often the senior clinical decision-maker on site, working with a team of care staff and with visiting professionals such as GPs (general practitioners — the UK's family doctors), district nurses, pharmacists and therapists.

Skills for Care, the workforce body for adult social care in England, describes the registered nurse role in social care as a safety-critical role rooted in nursing professional practice, supporting people with a range of complex needs — and notes that it carries a high degree of autonomy.

WHAT THIS ACTUALLY MEANS

Care settings differ from acute hospitals in rhythm rather than in seriousness. You are usually caring for the same people over months or years rather than days, which changes how you assess, plan and evaluate care. Clinical needs can be complex — long-term conditions, frailty, dementia, end-of-life care, tissue viability, nutrition, complex medication regimes. The immediate support around you is usually a smaller team than a large hospital puts on one site, which places more weight on your own assessment, escalation and documentation — and correspondingly gives you more room to exercise judgement than many acute roles do. Many internationally educated nurses find it a strong first UK role for exactly that reason; others prefer acute settings. Neither choice is a lesser one.

Community and other healthcare settings

A great deal of UK healthcare happens outside hospital buildings. Registered nurses work in district and community nursing teams, in GP practices and primary care, in hospices and palliative care, in mental health and learning disability services, in prisons, in schools, in occupational health, in clinical research and in telephone and digital triage services. Which of these are realistically open to you depends on your field of registration, your experience, your specialty and what a particular employer is recruiting for.

One register, four healthcare systems

The UK is made up of England, Scotland, Wales and Northern Ireland. Health policy is devolved, so each nation runs its own system, its own employers and its own job boards.

How healthcare is organised across the UK

NATION	HOW SERVICES ARE ORGANISED	WHERE JOBS ARE ADVERTISED
England	NHS trusts and foundation trusts deliver services; integrated care boards plan them locally. National oversight arrangements are currently being restructured.	jobs.nhs.uk, plus employers' own career pages
Scotland	NHS Scotland is organised around 14 territorial health boards, with additional national boards.	apply.jobs.scot.nhs.uk
Wales	NHS Wales delivers services through 7 local health boards and 3 NHS trusts.	jobs.nhs.uk (Wales advertises there too)
Northern Ireland	Health and social care are structurally integrated under Health and Social Care (HSC) trusts.	jobs.hscni.net

The one thing that does not change

The NMC is the independent regulator for nurses and midwives across the whole of the UK. One registration, one PIN, valid in all four nations. (The separate *nursing associate* role, which the NMC also regulates, exists in England only — it is a different registration from that of a registered nurse, and not the route this guide covers.)

What does change between nations

- **Pay and terms.** Each nation negotiates its own NHS pay scales. The figures differ, sometimes noticeably.
- **Employers and structures.** Trust, health board and HSC trust are not interchangeable words. Broadly: an NHS trust in England runs services and is commissioned to do so; a health board in Scotland or Wales both plans and runs services for its area; an HSC trust in Northern Ireland runs health and social care services together.
- **Where vacancies are published.** Scotland and Northern Ireland use their own recruitment systems, so searching only jobs.nhs.uk will hide a large number of roles from you.
- **Care regulators.** Services are inspected by the Care Quality Commission in England, the Care Inspectorate and Healthcare Improvement Scotland in Scotland, Care Inspectorate Wales and Healthcare Inspectorate Wales in Wales, and the Regulation and Quality Improvement Authority (RQIA) in Northern Ireland. Their published inspection reports are a genuinely useful way to research a prospective employer.
- **Cost of living and geography.** Salary is not the same as what you keep. Housing costs vary enormously across the UK, and so does public transport.

WHAT THIS ACTUALLY MEANS

Your NMC registration opens the whole UK to you. Your job search does not open itself — you have to go looking in four places, not one. Nurses who search only the England job board routinely miss roles in Scotland and Northern Ireland that would have suited them well.

What does a UK registered nurse role actually look like?

Whatever the setting, a UK registered nurse is an autonomous, accountable professional. You are not simply carrying out instructions — you are assessing, deciding, recording and answering for your own practice.

Responsibilities you can expect in almost any nursing role

- Assessing patients, residents or service users
- Planning and delivering care
- Monitoring health and wellbeing over time
- Administering medication safely and accurately
- Maintaining accurate, contemporaneous records
- Communicating with other healthcare professionals
- Communicating with families and carers
- Safeguarding adults and children
- Infection prevention and control
- Recognising deterioration
- Escalating concerns appropriately and promptly
- Supporting and working within multidisciplinary teams
- Maintaining professional standards and your own competence
- Educating and supporting patients and service users

The document that sits behind all of this

The NMC's *Standards of proficiency for registered nurses* set out what a registered nurse in the UK is expected to know and do. They are organised around seven platforms, from "being an accountable professional" through to "coordinating care". These standards also underpin the Test of Competence blueprint. If you read one UK professional document before you start preparing, read these — they tell you what the UK expects of you, which is a different question from what you were taught.

The exact responsibilities depend on where you work

Two nurses can hold identical registrations, identical job titles and identical pay bands and do noticeably different work. This is normal, and it is why reading the actual job description matters more than reading the title.

What you may commonly encounter, by setting

SETTING	WHAT YOU MAY COMMONLY ENCOUNTER
NHS hospital	Acute care, specialist departments, large multidisciplinary teams, formal escalation structures, high patient turnover, established policies and electronic systems
Private hospital	Similar clinical responsibilities depending on specialty, often with a narrower case mix, planned rather than emergency work, and employer-specific systems and terms
Nursing or care home	Long-term and continuing care, complex and multiple needs, medication rounds and reviews, care planning, end-of-life care, leading a care team, greater clinical autonomy
Community	Care delivered in people's homes, clinics or local services; lone working; travel between visits; assessment without immediate colleague support
Specialist setting	Role-specific clinical responsibilities — for example mental health, learning disabilities, palliative care, operating theatres, dialysis, occupational health or research

WHAT THIS ACTUALLY MEANS

None of these settings is a step up or down from the others. They are different kinds of nursing that suit different people, and a career can move between them. What matters is that the role matches your field of registration, your experience and the kind of work you want to do — and that you know what you are saying yes to before you accept it.

Band 5, and what nurses are actually paid

You will see "Band 5" in almost every NHS nursing advert, and internationally educated nurses usually meet the term before anyone explains it. It is worth understanding — but it only describes one part of the employment landscape.

Agenda for Change, briefly

Agenda for Change is the national pay and terms system used for most non-medical staff directly employed by NHS organisations — nurses, allied health professionals, healthcare scientists, administrative staff and many more. It groups roles into numbered bands based on job evaluation, and sets pay scales, annual leave, working hours and other terms for each band. Doctors and dentists sit outside it, on their own arrangements.

Each band has pay points, so pay rises with time in the role as well as with promotion to a higher band. Band 5 is the band most newly registered clinical professionals enter at, including newly registered nurses. Internationally educated nurses joining the NHS are very commonly recruited into Band 5 roles regardless of how many years they have practised at home, because the band reflects the job being done rather than the nurse's total career length.

Agenda for Change applies to the NHS — and only to the NHS

Independent hospitals, care home providers, hospices, GP practices, agencies and other non-NHS employers are **not** bound by Agenda for Change. Many set their own pay structures and terms. Some mirror NHS bands loosely; others do not use bands at all. If a non-NHS advert mentions a band, treat it as a rough comparison rather than a guarantee that NHS pay and terms apply. Sick pay, maternity pay, annual leave and pension arrangements are often different too — and the Royal College of Nursing notes that in the independent sector, pay rates are frequently lower than the NHS equivalent.

NHS Band 5 across the four nations

Each nation negotiates its own NHS pay scales, so the same band pays differently depending on where you work.

NHS Agenda for Change Band 5 annual pay, full time

NATION	ENTRY POINT	AFTER 2 YEARS	TOP OF BAND	EFFECTIVE
England	£32,073	£34,592	£39,043	1 April 2026
Scotland	£34,544	£36,911	£43,039	1 April 2026
Wales	£32,557	£35,114	£39,631	1 April 2026
Northern Ireland	£31,049	£33,487	£37,796	1 April 2025

Figures as published by the relevant health department or NHS body and checked in September 2026. Northern Ireland had not published a 2026/27 scale at that point, so the figures shown are its most recent published scale — check for a newer one. All figures are before tax, National Insurance and pension contributions, and exclude enhancements. Always check the current published scale for the nation and employer you are applying to.

Two things that change NHS take-home pay

Unsocial hours. Under the NHS terms in England, staff in Bands 4 to 9 receive an enhancement of time plus 30% for Saturdays and for weekday nights, and time plus 60% for Sundays and public holidays. A nurse working a lot of nights and weekends earns noticeably more than the basic scale suggests. Scotland, Wales and Northern Ireland have their own arrangements.

High cost area supplements. NHS roles in and around London attract a supplement on top of basic pay — broadly 20% in inner London, 15% in outer London and 5% in the surrounding fringe, each subject to a published minimum and maximum. It is not a bonus; it is there because living costs are higher, and it rarely covers the whole difference.

What about pay outside the NHS?

This is where most published guidance goes quiet, so it is worth being straight with you: for the private and social care sectors there is far less reliable public information about pay than there is for the NHS, and what does exist needs reading carefully.

How pay is set, sector by sector

SECTOR	HOW PAY IS SET	WHAT IS ACTUALLY PUBLISHED
NHS	National Agenda for Change scales, negotiated annually in each nation	Full pay scales published openly by each nation's NHS body. You can see exactly what a band pays before you apply.
Private / independent hospitals	Set by each employer. Some loosely mirror NHS bands; many do not use bands at all	No authoritative body publishes typical pay for this sector. You will only find out what a specific role pays by reading the advert or asking.
Nursing and care homes	Set by each provider, within whatever the provider can fund	Skills for Care publishes sector workforce data for England. Pay varies widely between providers and between regions — more so than in the NHS.
Community and primary care	NHS community services use Agenda for Change. GP practices and independent community providers set their own terms	Depends entirely on who the employer is. Check whether the advert is an NHS organisation or an independent one.
Hospices and charities	Set by each organisation; some voluntarily follow NHS scales	Usually stated in the advert. Ask directly whether NHS terms are matched, and whether that includes annual leave and pension.
Agency	Hourly rates, set by the agency and the assignment	Rates can look high per hour, but there is normally no guaranteed income, no occupational sick pay and no employer pension on the same terms. Agency work is also rarely a route to sponsorship.

The number that matters most if you need sponsorship

If you are coming to the UK on a sponsored work visa, there is a legal minimum your employer must pay you — and for registered nurses it is not set by the NHS. It is set by the immigration rules.

For healthcare jobs that sit on national pay scales, GOV.UK publishes a "going rate" by NHS band, and states that these going rates apply **whether you will be working in the public or the private sector**. Your salary must be at least that going rate, or a stated cash floor, whichever is higher. For a Band 5 registered nurse in England the published going rate is **£29,970**.

WHAT THIS ACTUALLY MEANS

This single figure explains a great deal about what you will see advertised. A nursing home or private hospital sponsoring an overseas nurse in England must pay at least around £29,970 — which is why so many sponsored roles outside the NHS cluster close to £29,000–£31,000, while an NHS Band 5 post starts at £32,073. The gap is lawful, and it is not a sign that anything is wrong. It simply reflects that the immigration floor sits below the NHS scale.

Two details worth knowing. First, the published going rates are pegged to an earlier year's NHS scales, so they fall further behind the live NHS scale each year. Second, they are a *minimum*, not a market rate — plenty of non-NHS employers pay well above them, and some pay very close to them. Ask what the salary is, in writing, before you accept anything.

Check these figures, do not rely on them

Salary thresholds, going rates and NHS pay scales are all revised, and not on the same timetable. Every figure on these two pages was checked against official publications in September 2026 and will date. Before you make a decision on the strength of a number, check the current NHS pay scale for the nation you are applying to, and the current going rate on GOV.UK.

Why the band or the salary never tells the whole story

- **Specialty changes the work.** Band 5 nurses in operating theatres, in a community team and in an acute medical service do very different jobs.
- **Setting changes the autonomy.** Support, supervision and escalation routes are not the same everywhere.
- **Shift patterns change the life.** Days, nights, long shifts, weekends and on-call arrangements vary by employer and role.
- **Enhancements change the pay.** Two nurses on identical basic pay can take home very different amounts depending on the hours they work and where they work.
- **Location changes what the money is worth.** A higher salary in an expensive city is not automatically a better offer.
- **The rest of the package matters.** Annual leave, sick pay, pension, and any accommodation or relocation support can be worth more than a difference of a thousand pounds in salary.

WHAT THIS ACTUALLY MEANS

Read the job description, not the band. The band tells you roughly where an NHS role sits in a pay structure, and outside the NHS it may tell you nothing at all. Before you accept anything, make sure you have seen the full job description and person specification, and that you have asked about salary, shift patterns, enhancements, annual leave, supervision and the induction you will receive — in writing.

Before you start: check whether you are ready

This is not a test and nobody is marking it. It is a way of finding out, in ten minutes, which parts of the journey you have already covered and which you have not thought about yet.

Ask yourself

- ☐ Am I a qualified nurse, with my qualification certificate available to me?
- ☐ Is my nursing qualification one that would allow me to register in the country where I trained?
- ☐ Do I hold, or have I held, professional registration with my home regulator — and can I evidence it?
- ☐ Can I obtain the documentation that will be requested: identity, qualification, registration, employment history, references, police clearance?
- ☐ Have I read the current NMC requirements on the NMC's own website, rather than only on social media or an agency page?
- ☐ Do I understand that the NMC's English requirement and the UK visa English requirement are set by two different organisations?
- ☐ Am I prepared to sit and pass the Test of Competence — both the computer-based test and the practical assessment?
- ☐ Am I prepared to practise within UK professional standards, including accountability, safeguarding and escalation as they are defined here?
- ☐ Do I have a realistic sense of the time and the cost involved, and a plan for both?
- ☐ Have I decided which field of nursing I intend to register in?

Two points worth knowing before you go further

You choose a field of registration, and it matters. The NMC registers nurses in specific fields — for example adult nursing, mental health nursing, learning disabilities nursing and children's nursing. Your qualification does not have to be specific to the field you apply for, but you must take the Test of Competence for the field you choose, and you cannot use the same pre-registration qualification to apply for more than one field. Think about this before you apply, not after.

The NMC needs your home regulator to confirm your position. The core eligibility test is whether your qualification has allowed, or would allow, you to register in the country where you trained. If you are already registered there, you provide the details and your certificate. If you never registered, the NMC will contact your regulator to confirm that your qualification would have allowed it. If your regulator cannot confirm this, the NMC may not be able to accept your application.

Verify your own eligibility with the NMC

Individual circumstances differ enormously — training routes, dates, registration history, gaps in practice, name changes, previous applications. No guide can tell you whether you personally are eligible. The NMC publishes the current requirements and an eligibility checking tool, and it is the only organisation whose answer counts. Start there.

IF YOU TICKED FEWER BOXES THAN YOU HOPED

That is useful information, not bad news. Almost every nurse begins this process with gaps. The ones who struggle are not the ones who started with gaps — they are the ones who did not know the gaps were there until an organisation asked them for something they could not produce.

NMC registration — start to finish

This is the longest and most detailed part of the journey, and the part where the most time is lost. The good news is that the NMC publishes the process openly, step by step, on its own website.

The shape of it: two applications, with an assessment in between

The overseas route is not one continuous form. You make an **eligibility and qualification application** first. Once the NMC has assessed your qualification, you work through your English language evidence and the **Test of Competence**. Then you make your **registration application**, bringing together your evidence of health, character, English and professional indemnity. Understanding that it is two applications, not one, makes the whole thing far easier to follow.



Step 1 — Create your NMC account

You begin with a route assessment: a short set of questions about your nationality, your training and the country you trained in, which confirms which route applies to you. You then create a MyNMC account, verified by email. This account is where your whole application lives — every upload, every status update, every message. Use an email address you will keep for years, not a work address you may lose.

WHAT THIS ACTUALLY MEANS

Creating the account is the moment the clock starts. The NMC closes applications that have not progressed within three months of being opened, and removes the online account. Do not open your account "just to look around" months before you are ready to work on it.

Step 2 — Submit the eligibility and qualification application

In this first application you provide your identity evidence — your passport details and a copy of the photograph page, with the passport valid at the time you upload it — and your qualification and registration details. You then pay the qualification evaluation fee. You cannot submit without paying it, and it is not refunded even if the NMC decides you are not eligible.

Step 3 — Verification of qualifications and registration

This is the part you do not control, and it is worth understanding properly. The NMC contacts your regulator directly to verify your position:

- **If you are already registered in the country where you trained** — you give the regulator's name and provide a copy of your registration certificate, and the NMC verifies it with them.
- **If you have never registered** — you name the regulator and the NMC asks them to confirm that your qualification *would* allow you to register with them. The NMC may also contact your education institution. If your regulator cannot confirm this, the NMC may not be able to accept your eligibility application.

The NMC aims to assess your qualification within 14 days — but that clock starts once it has received a response from your regulator, and the NMC does not control how long your regulator takes. For many nurses this is the single longest and least predictable wait in the whole process.

Step 4 — Evidence and supporting documents

Across the two applications, the NMC may ask for evidence including your identity documents, your qualification certificate, your registration certificate, evidence of any change of name, a health declaration, character evidence, police clearance certificates and your English language evidence. Chapter 10 covers each of these in detail, including who provides them and how long they take to obtain.

Two rules worth knowing now. First, any document not in English needs a full certified English translation, provided alongside the original and a stamped photocopy, with the translator's details. Second, at some point in the process every applicant must complete a **face-to-face identity check** at an NMC Test of Competence centre, where original documents are verified. On the overseas route this normally happens on the day of your first practical assessment.

Step 5 — English language evidence

Your English evidence forms part of your registration application. You can begin working towards it long before you get there, and most nurses should — it is the requirement that takes longest to put right if it goes wrong. Chapter 07 covers the detail, including the accepted tests, the exact scores, the rules on combining two sittings and the alternative routes that do not involve a test at all.

Step 6 — Test of Competence

The Test of Competence has two parts, and you can pass them in either order:

- **The CBT** — a computer-based test, sat at a test provider's centre. Centres are available in many countries, including India, so most nurses sit this before leaving home. Chapter 08.
- **The OSCE** — a practical, observed assessment, sat at an approved test centre in the UK. Chapter 09.

You must pass the CBT and the OSCE within two years of each other. Once you have passed both, your Test of Competence result remains valid for five years.

Step 7 — Complete remaining requirements

Your registration application brings together health and character evidence, your English language evidence, a declaration about professional indemnity arrangements, and your registration fee. The NMC then verifies your information, including contacting the referees you have nominated.

On indemnity, one point often misunderstood: you must have an indemnity arrangement in place to practise, but you do not personally have to hold the cover. For most employed nurses it is provided through their employer.

Step 8 — Registration

The NMC aims to consider a complete registration application within 30 days of having everything it needs, and is required to decide within three months of receiving a completed application. You are told the outcome by email.

Being on the NMC register means you are legally permitted to practise as a registered nurse anywhere in the UK, using a protected title, and that you are accountable to the NMC for your practice. You hold a PIN — your personal identification number on the register, which employers use to check your registration — you pay an annual retention fee, and in due course you will revalidate. What registration is *not* is a job, a visa, or a promise of either.

Deadlines that catch people out

Three months	An application that has not progressed within three months of being opened is closed, and the online account removed.
Two years	Once eligibility is confirmed you have two years to complete your registration, including both parts of the Test of Competence.
Two years apart	The CBT and OSCE must be passed within two years of each other.
Five years	How long a completed Test of Competence result stays valid once both parts are passed.
Two years (English)	Language test scores are valid for two years and must still be valid when your application is assessed.
28 days	The window to appeal a registration decision, on limited grounds only.

On cost

There are fees at several points: a qualification evaluation fee and a registration fee paid to the NMC, and separate fees paid to the test providers for the CBT and the OSCE. Once registered you pay an annual retention fee. The NMC publishes the current amounts on its "costs" page, and has announced changes to some of them taking effect from 1 October 2026 — so check the figures there rather than relying on any number you read elsewhere. None of these figures include your visa, flights, or accommodation and travel in the UK.

Not everyone's route is identical

The sequence above is the general shape of the overseas route. The exact requirements, order and evidence can differ depending on where you trained, when you qualified, your registration history, whether you have practised recently, and other personal circumstances. The NMC's own step-by-step guide for applicants trained outside the UK is the authority on what applies to you.

The English requirements — why this confuses so many nurses

There are two separate English requirements in this journey, set by two separate organisations, for two separate purposes. Treating them as one requirement is one of the most common and most expensive mistakes on the whole route.

Two requirements, side by side

	NMC	IMMIGRATION
Who sets the requirement?	The Nursing and Midwifery Council	The UK Government
Why is it required?	Professional registration — can you communicate safely and effectively as a registered nurse?	Immigration and visa requirements — do you meet the language condition of the route you are applying on?
What is measured?	Specific scores in listening, reading, writing and speaking, in accepted tests — or an accepted alternative form of evidence	A stated level on the Common European Framework of Reference (CEFR), proved in one of several accepted ways
What should nurses verify?	Current NMC guidance	Current GOV.UK guidance

Never assume one covers the other

Never assume that meeting one English requirement automatically means you have met every English requirement you may encounter during the UK journey. The two are set independently, they can change independently, and only the organisation that set a requirement can tell you whether you have met it.

The NMC requirement

The NMC accepts three stand-alone forms of English language evidence:

- **An accepted English language test** with the required scores, achieved recently enough to still be valid when your application is assessed.
- **A pre-registration nursing qualification taught and examined in English.** Where the programme was not NMC-approved, this is evidenced through your transcript, which must show that you spent at least half your time interacting with patients, service users, families and other healthcare professionals, and that at least 75% of those interactions were in English.
- **One year of recent practice in a country where English is the majority spoken language,** as a registered professional. The NMC publishes a closed list of accepted countries. India is not currently on that list, so this route is only relevant to nurses who have practised in one of the listed countries. The list is revised from time to time — check the current version on the NMC's website.

There is also **SIFE** — Supporting Information From Employers — which is a form of structured employer reference. It is important to understand what SIFE is not: it is never stand-alone evidence. It can only be used *alongside* another form of evidence, it requires at least twelve months' employment with a UK health or care employer, and it is therefore not a route available to a nurse still in India.

WHAT THIS ACTUALLY MEANS FOR MOST INDIAN NURSES

Two realistic routes. Either you sit an accepted test and achieve the required scores, or — if your nursing programme was taught and examined in English, as many Indian programmes are — you evidence that through your transcript. It is worth checking early whether your institution can produce a transcript in the detail the NMC asks for, because obtaining it can take time and many nurses assume it will be straightforward.

The accepted tests and scores

The NMC accepts two tests: **IELTS Academic** and the **Occupational English Test (OET)**. The General Training version of IELTS is not accepted. The required scores as published by the NMC are:

NMC English language test requirements

SUB-TEST	IELTS ACADEMIC	OET
Listening	7.0	Grade B
Reading	7.0	Grade B
Speaking	7.0	Grade B
Writing	6.5	Grade C+

Scores as published by the NMC and checked in September 2026. Requirements can change — confirm the current scores on the NMC's accepted English language tests page before booking anything.

Combining two sittings

The NMC allows you to combine results from two sittings, which has rescued a great many applications. The rules are strict and all of them must be met:

- Both sittings must be of the **same test type** — you cannot combine an IELTS result with an OET result.
- The two sittings must be taken **within 12 months of each other**, and both must still be within the two-year validity window when your application is assessed.
- Each sitting must have **tested all four skills at the same time** — you cannot retake a single component in isolation.
- You must achieve a required pass score in each of the four skills **across the two sittings**.
- You must not fall below the NMC's minimum floor scores in either sitting. The NMC publishes these floors and provides a test-combining calculator on its website.

WHAT THIS ACTUALLY MEANS

If you are close but not there, do not panic and do not assume you must start again. Read the combining rules carefully and use the NMC's own calculator. But do read them *before* you book your second sitting, because a sitting taken at the wrong time, or in the wrong test, cannot be combined afterwards.

The immigration requirement

Separately, work visa routes set their own English language condition, expressed as a level on the CEFR scale. GOV.UK lists the ways it can be proved — which include an approved Secure English Language Test, certain UK qualifications, a degree taught in English (with an assessment of a non-UK degree by the designated body), and an exemption for nationals of certain listed countries. Indian nationals are not currently on that exemption list — but the list can change, so check the current one on GOV.UK.

One point that often helps nurses

GOV.UK states that you do not need to prove your knowledge of English separately if you have already passed an English language assessment accepted by the relevant regulated professional body — and it lists nurses among the professions this applies to. In practice this means the English test you take for the NMC may also satisfy the visa requirement. It does not mean the two requirements are the same thing: it means one route of proof happens to serve both. Check the current position on GOV.UK when you apply, and never assume it applies to your case without checking.

Before you book a test

Check on the NMC's website which test versions and delivery formats it currently accepts, and check on GOV.UK which providers and test versions are approved for visa purposes. A test booked in the wrong version, or with the wrong provider, is time and money you cannot get back.

CBT stands for Computer Based Test. It is the first of the two parts of the NMC's Test of Competence, and for most nurses in India it is the first real hurdle of the whole journey.

Where it fits

The CBT sits inside the Test of Competence, alongside the practical OSCE. You can pass the two parts in either order, but the CBT is the one you can sit without leaving India — the NMC's test provider operates centres in many countries — so the great majority of nurses take it first, while their NMC application is progressing.

What it assesses and how it is structured

The CBT is taken in a single sitting made up of two parts, which are marked independently of each other. If you pass one part and not the other, you only resit the part you failed.

CBT structure for nurses

PART	WHAT IT COVERS	QUESTIONS	TIME
Part A	Numeracy	15	30 minutes
Part B	Clinical questions for your field of nursing	100	2 hours 30 minutes

3

attempts per application, with at least 10 days between each sitting

1 year

within which all three attempts must be taken

48 hrs

the NMC's stated timescale for results appearing in your account

What happens if you use all three attempts

If you are unsuccessful at the third attempt, your application closes and you must wait six months before submitting a new application. This is why "book it and see how it goes" is a poor strategy. Treat every attempt as if it were the only one.

Structure, attempt limits and timescales as published by the NMC and checked in September 2026. These can change — confirm the current position on the NMC's CBT page before you book.

ON PASS MARKS

The NMC does not publish a numeric pass mark for the CBT. It explains that a standard is set and maintained statistically across different versions of the test. Any specific percentage you see quoted online is not an NMC figure. Prepare to be comfortably competent rather than to clear a number.

How to prepare

The most common mistake here is preparing as though it were an examination about nursing facts. It is an assessment of whether you can practise safely and reason clinically *to UK standards*. Focus your preparation on understanding:

- **UK professional expectations** — accountability, consent, capacity, confidentiality, record-keeping, delegation, raising concerns
- **Clinical reasoning** — not just what the right answer is, but why the other options are wrong
- **Safe practice** — medicines safety, infection prevention, recognising deterioration, escalation
- **The relevant NMC standards** — The Code, and the Standards of proficiency for registered nurses
- **The current Test of Competence blueprint** for your field, which the NMC publishes and which maps exactly what can be assessed

For Part A, practise UK drug calculations specifically — units, formulations, infusion rates and conversions as they are expressed in UK practice. Accuracy under time pressure matters more than speed.

CBT preparation checklist

- ☐ Read the current NMC guidance and candidate information for the CBT
- ☐ Download and work through the Test of Competence blueprint for your field
- ☐ Read The Code and the Standards of proficiency for registered nurses
- ☐ Build your UK terminology — the words used here for drugs, grades of staff, documents and escalation routes
- ☐ Practise questions regularly rather than intensively for a short period
- ☐ Review every incorrect answer until you can explain why it was wrong
- ☐ Practise under timed conditions, including Part A's 30 minutes
- ☐ Practise numeracy separately and daily — it is the part most often failed and most easily fixed
- ☐ Only book your sitting when your practice results are consistently strong

The Test of Competence & OSCE

The Test of Competence consists of the CBT and OSCE components.

They are two halves of one assessment, and you are not finished until you have passed both.

What OSCE means

OSCE stands for **Objective Structured Clinical Examination**. Instead of answering questions about nursing, you demonstrate nursing — in a simulated clinical environment, observed and marked by examiners against published criteria. "Objective" and "structured" are the important words: every candidate faces the same tasks and is marked against the same criteria, so the assessment is as consistent as it can be made.

Why it is used

A written test can show that you know what should happen. It cannot show whether you would actually do it, in the right order, safely, while explaining yourself to a patient and documenting it properly. The OSCE exists to test the gap between knowing and doing.

How it is structured

The OSCE for nurses is made up of **ten stations**:

- **Four linked stations** built around a single patient scenario, following the nursing process — assessment, planning, implementation and evaluation, commonly referred to as **APIE**. One station for each stage.
- **Four skills stations**, arranged as two pairs of two skills.
- **Two further stations** assessing professional values and behaviours, and evidence-based practice.

Across those ten stations, the assessment is designed to look at your clinical skills, your communication, the safety of your practice, your professional judgement, whether your care is genuinely centred on the person in front of you, how you document what you do, and whether you escalate appropriately when something is wrong.

Where and how often

The OSCE is sat in the UK at an approved test centre. The NMC publishes the current list of centres, and that list changes — two centres closed during 2026, so check which are operating before you plan anything around a location.

You have a maximum of three attempts within one application, with at least ten days between sittings, and resits must be taken at the same centre as your first attempt. If you fail at the third attempt you must wait at least six months and then retake all ten stations under a new application. There is a reduced fee for a partial resit where you are only resitting some stations.

Check the current station list before you prepare

The NMC periodically refreshes the scenarios and skills that can appear, publishing the changes in advance on its Test of Competence resources page — a set of new stations was announced for candidates sitting from late October 2026, for example, without changing the number of stations. Do not rely on a station list from a coaching centre, a video, or a nurse who sat the OSCE last year. Use the NMC's own current materials: the marking criteria, station timings and equipment lists for your field are all published there.

A seven-step preparation plan

Start earlier than feels necessary. Nurses rarely fail the OSCE because they are poor nurses. They fail because they prepared for a different examination from the one they sat.

1 Understand the assessment

Read The NMC's current OSCE materials for your field before anything else — the marking criteria, the station timings, the equipment list, the blueprint. Know exactly what is being marked and how the marks are awarded. Most preparation is wasted because this step is skipped.

2 Learn the UK approach

Read The Code and the Standards of proficiency, and learn how the UK frames consent, capacity, dignity, confidentiality, safeguarding, infection prevention, medicines safety, escalation and documentation. Much of this is about the way care is given, not the clinical content. This is where internationally educated nurses most often lose marks.

3 Practise the skills

Physically, with your hands, using the equipment on the published list where you can. Reading about a skill and performing it under observation are not the same activity. Practise the full sequence including preparation, hand hygiene, checking, the skill itself, and what you do afterwards.

4 Practise explaining what you are doing

Out loud, in English, to the person you are caring for. Examiners cannot mark what they cannot observe, and communication is assessed in its own right. Practise introducing yourself, gaining consent, explaining what you are about to do and why, checking understanding, and closing the interaction.

5 Practise with another person

Ideally another nurse, ideally one who will be honest with you. Take turns as candidate and observer. Mark each other against the published criteria rather than on general impression.

6 Simulate the assessment environment

Timed, uninterrupted, standing up, with equipment you have to find and prepare yourself, and with someone watching. The pressure of being observed is itself a skill you can practise, and the timing catches people out far more than the clinical content does.

7 Review feedback

After each practice run, identify precisely what went wrong and why — a missed step, a safety omission, poor sequencing, unclear communication, incomplete documentation, or simply running out of time. Fix one thing at a time. Repeat.

THE MOST IMPORTANT THING TO UNDERSTAND ABOUT OSCE PREPARATION

Preparation should be about safe clinical reasoning and professional practice, not about memorising scripts. Candidates who learn a sequence of words by heart come apart the moment a station differs slightly from the one they rehearsed — and stations do change. Candidates who understand *why* each step exists can adapt, because they are thinking like a registered nurse rather than reciting. Examiners can tell the difference easily.

Support from your employer

Many UK employers who recruit internationally educated nurses provide structured OSCE preparation after arrival, alongside accommodation and pastoral support. NHS Employers publishes guidance encouraging exactly this. When you are talking to a prospective employer, what OSCE support they provide is a fair and sensible question to ask — and the answer tells you a lot about the organisation.

Your documents — what to start collecting now

The NMC's own advice to applicants is to gather evidence *before* starting an application, so there are no unnecessary delays. It is sound advice, and it is easy to skip.

The order below is roughly the order in which you are likely to be asked for things. Your own order will differ — some employers ask for references early, some visa evidence is only needed at the end, and some documents are requested more than once by different organisations.

Document by document

DOCUMENT	WHAT IT IS AND WHY IT MATTERS	WHO MAY REQUEST IT	WHERE TO OBTAIN IT
Passport	Your primary identity and nationality document. Almost everything else attaches to it. Must be valid, and valid well beyond your planned travel.	NMC, employer, Home Office	Passport authority in India. Renew early if it expires within the next two years.
Nursing qualification certificate	Proof of the qualification you are applying on. Must show your name, the qualification title and the date of award.	NMC, employer	Your university or nursing college. Replacements can take months.
Academic transcripts	The detailed record of your programme — hours, theory, clinical placements, language of instruction. Essential if you are evidencing English through your qualification.	NMC	Your institution. Ask specifically for the level of detail the NMC requires.
Professional registration certificate	Evidence of your registration with your home regulator, which the NMC then verifies directly with them.	NMC, employer	Your state nursing council or national regulator.
Evidence of any name change	Reconciles a different name on your certificates, registration or passport — for example after marriage.	NMC, employer, Home Office	Marriage, civil partnership or divorce certificate, or a deed poll document (the UK's formal document for changing a name).
Employment history	A complete, dated record of where you have worked, in what role, with no unexplained gaps.	Employer, NMC	Build it yourself now, and verify dates against payslips or contracts.
References	Professional references from people who can speak to your practice and conduct. Character evidence may come from your regulator, an employer, an institution or an NMC registrant depending on your circumstances.	Employer, NMC	Current and previous managers. Ask permission before naming anyone, and confirm their correct email address.
English language evidence	Your test result, or the transcript evidence for the qualified-in-English route.	NMC, Home Office	The test provider, or your institution. Note the two-year validity on test scores.
Police clearance certificates	Evidence of your criminal record position. The NMC asks for clearance from each overseas country you have lived in for twelve months or more, cumulatively, in the last ten years. Certificates must be recent.	NMC, Home Office, employer	The relevant police authority in each country. This is frequently the slowest document to obtain.
Health declaration	Confirmation from a registered medical practitioner that you are in good health for the purposes of registration, based on an assessment within the last twelve months.	NMC	A doctor or occupational health practitioner — not a friend or family member.
Occupational health information	Immunisation and health clearance required by an employer before you start work.	Employer	Your own records, plus the employer's occupational health service after you are appointed.
Job offer documentation	Your offer letter, contract and job description — the record of what you have actually agreed to.	You, Home Office	Your employer. Read all of it before signing, and keep a copy.
Sponsorship documentation	Your Certificate of Sponsorship reference and the details recorded on it, which you use in your visa application.	Home Office	Your sponsoring employer.
Immigration documents	Everything supporting your visa application, and afterwards the evidence of your immigration status.	Home Office, employer	GOV.UK sets out exactly what is required for the route you apply on.

Documents that can take time

Some documents arrive in an afternoon. Others depend entirely on an organisation that has no interest in your timeline. These are the ones that regularly cost nurses months:

- **Police clearance certificates**, particularly from more than one country
- **Detailed academic transcripts**, especially from an institution you left years ago
- **Replacement certificates** for anything lost or damaged
- **Your regulator's response** to the NMC's verification request
- **Certified translations** of any document not in English
- **References** from a previous employer or manager you have lost touch with
- **Passport renewal**, if yours is close to expiry
- **English test dates**, where centres are busy

WHY YOU SHOULD START BEFORE ANYONE ASKS

There is no advantage in waiting to be asked. Every one of these can be obtained now, sitting at home, while nothing else is happening. Nurses who collect documents only when an organisation requests them end up waiting twice: once for the request, and once for the document. Nurses who collect them in advance simply upload and move on.

Your document tracker

Print this page, or copy it into a spreadsheet. Keep one row per document and update it as you go. Add rows for anything specific to your own circumstances.

DOCUMENT	HAVE IT?	VERIFIED?	UPLOADED / SUBMITTED?	NOTES
Passport				
Nursing qualification certificate				
Academic transcripts				
Registration certificate				
Name change evidence				
Employment history				
References				
English language evidence				
Police clearance (country 1)				
Police clearance (country 2)				
Health declaration				
Certified translations				
Job offer and contract				
Certificate of Sponsorship details				

Three habits worth building

Keep originals physically safe and together. Keep a clear scan of every document in secure cloud storage you can reach from any country. And keep a copy of everything you submit to anyone — you will be asked for the same document more than once, often by a different organisation, sometimes a year later.

Finding a UK nursing role

Registration is a professional milestone. It is not a job. Finding the right role is a separate piece of work, and it rewards the nurses who treat it as one.

Where roles are advertised

Routes into a UK nursing role

ROUTE	WHAT IT IS AND WHAT TO KNOW
NHS Jobs	The official job site for NHS vacancies in England and Wales, at jobs.nhs.uk . Thousands of posts are advertised every month. You can search by role, band, location and employer.
Scotland and Northern Ireland	NHS Scotland, and Northern Ireland's HSC trusts, use their own recruitment systems. If you search only jobs.nhs.uk , you will never see those vacancies.
NHS trust and health board websites	Many organisations run their own careers pages, sometimes with international recruitment pages explaining the support they offer.
Private healthcare employers	Independent hospital groups and healthcare providers advertise on their own sites and on general job boards.
Nursing and care home employers	Care providers range from single homes to large national groups. Larger providers often have structured international recruitment programmes.
Recruitment agencies	Agencies can introduce you to employers and support the process. Check who they actually work with, what they will do, and be clear about anything you are asked to pay for.
Professional networks	Former colleagues already working in the UK, alumni groups and professional bodies. Informal, but a real source of accurate information about what an employer is actually like.

Ethical international recruitment

The UK Government publishes a *Code of Practice for the international recruitment of health and social care personnel*, setting standards for how employers and recruitment organisations should behave, and grading countries red, amber or green for active recruitment. At the time of writing India is graded green, with a government-to-government agreement in place. The grading list is revised, so check the current version. Reading the Code is worthwhile: it tells you what good practice looks like, which makes it easier to recognise when something falls short of it.

What to look for in a vacancy

UK job adverts are usually detailed, and the detail is there to be read. Before applying, make sure you can answer all of the following from the advert and the attached documents:

☐ Job title — and what it means in practice	☐ English requirements — what the employer asks for
☐ Employer — the actual legal organisation	☐ Sponsorship information — whether the role is open to applicants needing sponsorship
☐ Location — town or city, and the specific site	☐ Salary — the range, and whether it includes enhancements
☐ Setting — hospital, care home, community, other	☐ Working hours — full time, part time, contracted hours
☐ Specialty — and whether it matches your experience	☐ Shift patterns — days, nights, weekends, long shifts
☐ Required experience — and whether yours qualifies	☐ Training and support — induction, preceptorship, OSCE support
☐ Registration requirements — NMC registration, or willingness to consider applicants awaiting it	☐ Application process — closing date, format, what to submit

Read the job description, not the job title

Every "Registered Nurse" role is not the same role. Two adverts with identical titles and identical bands can describe completely different jobs in completely different settings with completely different shift patterns. UK employers publish a full job description and person specification for a reason. Download both, read both, and check your experience against the person specification line by line before you apply — that document is usually exactly what you will be scored against.

Practical points about applying

- **Applications are usually structured forms, not CVs.** Many UK employers, particularly in the NHS, ask you to complete an online application including a supporting statement, rather than attaching a CV.
- **The supporting statement is where applications are won and lost.** Address the person specification directly, point by point, with evidence from your own practice.
- **Closing dates are real.** Popular vacancies sometimes close early once enough applications are received.
- **Apply for roles you can actually do.** A carefully targeted application to a suitable role beats twenty generic ones, and employers notice the difference immediately.
- **Check the employer.** Inspection reports from the relevant care regulator are published and free to read. So are staff survey results for NHS organisations.

What UK employers look for

Employers are not simply checking that you hold a nursing qualification. They are assessing whether you will practise safely and well in *their* setting, within UK professional standards, from a reasonably early point.

Twelve things employers assess

WHAT THEY ASSESS	WHAT THEY ARE ACTUALLY LOOKING FOR
Clinical experience	Relevance rather than years. What you have actually done, with which patient groups, at what level of responsibility.
Communication	Whether you can explain, listen, check understanding and be understood — with patients, families and colleagues alike.
Patient-centred care	Whether care is built around the person's needs, preferences, dignity and consent, rather than around the task list.
Professionalism	Accountability, honesty, reliability, and understanding the limits of your own competence.
Safeguarding	Whether you can recognise when an adult or child may be at risk, and know that you must act and report.
Medication safety	Checking, calculating, administering, recording — and what you do when something is wrong.
Infection prevention	Consistent, routine practice, not something remembered when observed.
Escalation	Recognising deterioration and knowing when, how and to whom to raise it. Employers value a nurse who escalates early far above one who waits.
Teamwork	Working within a multidisciplinary team, supporting colleagues, accepting support, and handing over well.
Documentation	Accurate, timely, clear records. In the UK, if it is not recorded, it is treated as not done.
Professional standards	Familiarity with The Code and the expectations placed on a UK registered nurse.
Adaptability	Willingness to learn local systems, ask questions and work differently from how you were trained — without losing your clinical judgement.

WHAT THIS ACTUALLY MEANS

Many internationally educated nurses arrive clinically strong and are still marked down at interview, because they describe what they did without ever explaining how they thought, why they acted, or what standard they were working to. UK recruitment is heavily weighted towards judgement, safety and values. Your clinical skill is assumed to be there — what is being tested is how you use it.

How to make your experience understandable to a UK employer

A UK recruiter may not know your employer, your grading system, your patient volumes or what your job title means. That is not a disadvantage — it simply means the work of explaining is yours.

How to describe what you do

INSTEAD OF	EXPLAIN	EXAMPLE OF HOW TO PUT IT
Your current role	What you are responsible for, who you report to, who reports to you, and what you decide on your own	"I am a staff nurse in a 30-bed medical unit. I take a caseload of 8 to 10 patients per shift, and I lead the shift for a team of four when the senior nurse is off duty."
"Medical unit"	The patient population: age range, conditions, acuity, dependency	"Mostly adults over 60 with diabetes, COPD, heart failure and post-stroke rehabilitation needs. Around a third need full assistance with personal care."
"All nursing duties"	Specific responsibilities, with the scope you hold	"Full assessment on admission, care planning, IV and oral medication administration, wound care, venepuncture and cannulation, discharge planning with the family."
"Good clinical exposure"	Procedures and equipment you use routinely, and how often	"Daily use of infusion pumps and syringe drivers, weekly management of central lines, routine ECG recording and interpretation of basic rhythms."
"Senior nurse"	The leadership you actually exercise	"I mentor two student nurses each rotation, allocate the workload for my bay, and act as shift coordinator roughly twice a week."
"Specialised in..."	The specialty, the duration, and the competencies you hold	"Three years in intensive care, including ventilated patients, with unit-assessed competence in airway management and arterial line care."
"Awards and achievements"	What changed because of you	"I led a change to the handover process on my unit, which reduced missed medication doses — we audited it over three months."

Three rules for translating your experience

Use numbers. Bed numbers, caseload sizes, staffing ratios, frequency. Numbers travel across health systems in a way that job titles do not.

Use UK terms where you know them, and explain where you do not. If your job title has no UK equivalent, describe the job rather than translating the title into something that may mean the wrong thing.

Say what you decided, not only what you did. "I noticed, I assessed, I escalated, I documented" tells a recruiter far more than a list of tasks.

13 INTERVIEWS

UK nursing interviews

UK nursing interviews are usually structured, often panel-based, and increasingly held online for international candidates. They tend to be less about testing knowledge than about understanding how you think and what you value.

The formats you are likely to meet

Common question types

TYPE	WHAT IT IS TESTING
Competency questions	Evidence from your real experience. "Tell us about a time when..."
Scenario questions	Your judgement in a hypothetical situation. "What would you do if..."
Clinical questions	Your clinical knowledge and reasoning, usually relevant to the specific role
Values-based questions	Whether your values match the organisation's — dignity, compassion, respect, honesty
Safeguarding scenarios	Whether you recognise risk and know that you must act, report and record
Communication scenarios	How you handle difficult conversations with patients, families or colleagues
Teamwork questions	How you work with others, and how you handle disagreement or conflict

Structuring your answers: STAR

For competency questions, UK interviewers are usually listening for a clear structure. The most widely used is **STAR**:



Keep the Situation and Task brief — one or two sentences each. The Action is where the marks are, and it must be what *you* did, not what the team did. Finish with the result and, where honest, what you took from it.

Questions you may well be asked

"Tell us about a time you dealt with a difficult situation."

They are listening for: a real example, your specific role in it, how you weighed the options, whether you involved the right people, and whether you reflected afterwards. They are not listening for a situation in which you were heroic and alone. Choose something genuine and reasonably recent, and be honest about what was hard.

"Tell us about a time you recognised a deterioration in a patient or service user."

They are listening for: what you observed and measured, how you interpreted it, what you did first, who you escalated to and how quickly, what you documented, and what the outcome was. Be specific about the observations. Say clearly that you escalated — escalating is the right answer in the UK, never a sign of weakness.

"What would you do if you were concerned about a colleague's practice?"

They are listening for: that you understand you have a professional duty to act and cannot ignore it; that your first concern is the safety of the person receiving care; that you would raise it through the proper route with your line manager or safeguarding lead; that you would record it factually; and that you understand a concern is not an accusation. Do not answer that you would speak to the colleague and leave it there.

There is no perfect answer, and panels can tell when you are reciting one

Model answers learned from the internet are obvious to an experienced panel, because they contain no real detail and fall apart under a follow-up question. Panels almost always ask follow-up questions. Prepare your *examples*, not your *script*: think of six to eight genuine situations from your own practice, cover each in the STAR structure, and be ready to adapt them to whatever you are actually asked.

Practical preparation

- **Prepare six to eight real examples** spanning deterioration, a difficult conversation, a mistake or near miss, teamwork, leadership, a safeguarding concern and a time you had to learn something quickly.
- **Read the person specification again** the day before — the questions are usually drawn from it.
- **Research the employer**: their values, their setting, their inspection reports.
- **Test your technology** if the interview is online. Check your camera, microphone, internet connection, lighting and background, and have a phone number to call if it fails.
- **Prepare your own questions**: induction, preceptorship, OSCE support, supervision, shift patterns, the team you would be joining.
- **Be honest about gaps**. "I have not done that, but here is the closest thing I have done, and here is how I would approach learning it" is a strong answer. Pretending is not.

14 SPONSORSHIP

Sponsorship — in plain English

Six words get used almost interchangeably in conversations about moving to the UK, and they mean six different things. Getting them straight protects you from a great deal of confusion — and from people who benefit from that confusion.

Six words, six meanings

Job	The actual work: a nursing role, in a particular setting, for a particular employer, with particular hours and duties.
Job offer	A decision by an employer to give you that job, normally in writing and normally subject to conditions such as references, checks and clearances.
Employer	The organisation that employs you, pays you and is responsible for your working conditions.
Sponsor	An employer that holds a licence from the Home Office allowing it to sponsor workers from overseas. Not every employer holds one, and holding one does not oblige an employer to sponsor any particular role.
Certificate of Sponsorship (CoS)	An electronic record — not a paper document — that a licensed sponsor assigns to you for a specific job. It carries a reference number, the job title, the salary, the occupation code, the employer's name and their sponsor licence number.
Visa	Permission from the UK Government to enter and stay in the UK for a specified purpose. Granted by the Home Office after assessing your application. Nobody else can grant it.

How the pieces connect



WHAT THIS ACTUALLY MEANS

Each arrow is a separate decision, made by a different party, against different criteria. An employer deciding to sponsor you does not decide your visa. A Certificate of Sponsorship is not a visa and does not guarantee one — it is a required input to an application that is then assessed on its own merits. And timing matters: a certificate must be used within a limited period after it is assigned, and an application cannot be made too far ahead of the job's start date. GOV.UK sets out the current rules.

Things you should verify for yourself

- ☐ **The identity of the employer** — the full legal name of the organisation, and that it exists at the address given
- ☐ **The job you are being offered** — the title, the setting, the location, the hours, the shift pattern and the salary, in writing
- ☐ **The sponsorship arrangements** — whether the employer is a licensed sponsor, and whether this role is being sponsored
- ☐ **The applicable visa route** — which route you would be applying on, and its current requirements
- ☐ **The official documentation** — that the details on your Certificate of Sponsorship match the job you accepted

How to check whether an employer can sponsor

You do not have to take anyone's word for it. The Home Office publishes a **register of licensed sponsors** on GOV.UK, updated regularly. Search it for the employer's exact legal name and check that they appear with a Worker licence. You can also check that the sponsor licence number quoted on your Certificate of Sponsorship matches the register entry. It takes a few minutes and it is entirely free.

Verify important information through official sources

Whoever is helping you — an employer, a recruiter, a friend, a guide like this one — check anything that matters against the organisation that actually decides it. Registration questions: the NMC. Immigration questions: GOV.UK. If you need advice about your own immigration circumstances rather than general information, that advice must come from an appropriately regulated adviser.

The visa and immigration stage

This chapter explains the general shape of the process, so you know what to expect and what to check. It is not advice about your own case, and it cannot be — only GOV.UK, and where necessary a regulated adviser, can tell you what applies to you.

Step 1 — Identify the correct visa route

Registered nurses coming to work in the UK normally apply on the **Skilled Worker** route. GOV.UK also describes the **Health and Care Worker visa**, which is applied for as part of the Skilled Worker application for people doing eligible jobs with the NHS, an organisation providing medical services to the NHS, or an organisation providing adult social care. Registered nursing occupations are listed among the eligible jobs.

Routes change. In July 2025, for example, the UK closed new overseas applications in the care worker and senior care worker occupation codes — a change that applies to those specific codes and not to registered nursing roles. This is exactly why the route you are applying on, and the occupation code recorded on your Certificate of Sponsorship, are worth checking rather than assuming.

Step 2 — Certificate of Sponsorship

Your sponsoring employer assigns you a Certificate of Sponsorship and gives you its reference number, along with the details recorded on it. Check those details against your job offer before you apply, particularly the job title, occupation code, salary and start date.

Step 3 — Complete the application

You apply online through GOV.UK, creating or signing in to a UK Visas and Immigration account. You can save the form and return to it. You will be asked for your Certificate of Sponsorship reference and a range of personal and employment details.

Step 4 — Provide the required evidence

GOV.UK publishes the full list of documents for each route. For a Skilled Worker application it commonly includes your Certificate of Sponsorship reference number, a valid passport or other identity document, proof of your knowledge of English, the job details recorded on your certificate, evidence relating to funds unless your sponsor certifies maintenance, a tuberculosis test certificate if you have lived in a country on the UK's TB testing list, a criminal record certificate where required for the occupation, and certified translations of anything not in English.

Step 5 — Identity and biometrics

You will either verify your identity using the UK Immigration: ID Check app, or attend a visa application centre to have your fingerprints and photograph taken. Which applies depends on your circumstances and is confirmed during the application.

Step 6 — Wait for a decision

GOV.UK publishes expected processing times — for applications from outside the UK the published service standard is normally around three weeks once you have applied, proved your identity and provided your documents. Faster decisions may be available at additional cost. This is a service standard, not a promise, so do not book flights against it.

Step 7 — Receive the outcome

You are notified of the decision. Immigration status in the UK is now held digitally as an eVisa, accessed through your UKVI account, rather than as a physical document. You prove your status by generating a share code from that account — so keep the login details and recovery options for that account safe and accessible.

Immigration rules change, sometimes quickly

Salary thresholds, eligible occupations, English requirements, fees and processes are all reviewed and revised. Anything you read anywhere — including here — may have changed by the time you apply. Always work from the current GOV.UK guidance for the route you are applying on, and check it again shortly before you submit.

Many nurses complete this application themselves, following the steps on GOV.UK — it is designed to be done by the applicant, and doing it yourself keeps you in control of your own evidence. Others prefer to use an immigration adviser or a visa service to keep the process running smoothly, and some employers arrange that support as part of recruiting you. Both are legitimate choices. If you do use someone, check that they are appropriately regulated, be clear about what they will and will not do, and make sure you still hold copies of everything submitted in your name.

THREE QUESTIONS TO HOLD ONTO

What happens? A sequence of separate steps, each with its own requirements. **What documents are involved?** The ones GOV.UK lists for your route, plus certified translations of anything not in English. **What should I check?** That the route is right, that the details on your Certificate of Sponsorship match your offer, that your evidence is complete and current, and that you are reading today's guidance rather than last year's.

Preparing to move to the UK

Once the professional and immigration pieces are moving, the practical preparation begins. This is the part nurses most often leave until the last fortnight, and most often regret leaving.

Before you leave India

- ☐ **Passport and documents** — originals gathered, organised and carried in your hand luggage, never in a checked bag
- ☐ **Digital document backups** — clear scans of everything, in secure cloud storage you can access from any country, plus a copy with someone you trust
- ☐ **Employment documents** — your offer letter, contract, job description and your employer's contact details, printed and saved
- ☐ **Accommodation** — confirmed for your arrival, with the full address, how to get in, and who to contact if something goes wrong on the day
- ☐ **Travel arrangements** — flights, airport transfer, and a plan for arriving late at night or with delayed baggage
- ☐ **Banking arrangements** — how you will access money in your first weeks, and what you need to open a UK bank account
- ☐ **Phone and SIM** — a working number on arrival, and your Indian number kept active long enough to receive verification codes
- ☐ **Emergency contacts** — family in India, your employer, your accommodation, and someone in the UK if you have anyone
- ☐ **Health information** — a summary of any medication or health needs, carried with you, plus prescriptions as appropriate
- ☐ **Contact details for your employer** — including a named person and a number that works outside office hours
- ☐ **Understand how to get to your place of work** — the route, the journey time, and the alternative if transport fails
- ☐ **Weather and clothing** — the UK is colder and wetter than most of India for most of the year; arrive with a warm coat

Keeping documents safe

Keep originals together, in a single waterproof folder, in your hand luggage. Keep a full set of scans in cloud storage with two-factor authentication you can still access after changing your phone number. Keep a third copy — even photographs on a family member's device — somewhere entirely separate. Replacing a lost qualification certificate or registration document from overseas can take months, and you will need these documents again in the UK for employment checks and for your own records.

Questions worth asking your employer before you fly

What accommodation is arranged, for how long, and what does it cost? Who is meeting me, and where? What happens if my flight is delayed? When is my first working day? What do I need to bring for my identity and right-to-work checks? What OSCE support is provided, and when? Who is my named contact in my first week? Employers who recruit internationally are used to these questions, and a good one will have clear answers ready.

17 ARRIVAL

Arriving in the UK

Your first days will be busier administratively than clinically. Most employers front-load the checks and training, because you cannot start practising until they are complete.

What the first days and weeks commonly involve

Typical onboarding steps

STEP	WHAT IT USUALLY INVOLVES
Employer induction	Welcome to the organisation, its values, structure, policies and the practicalities of working there
Identity checks	Verification of your identity, right to work and immigration status, usually with original documents
Occupational health	A health assessment and review of immunisations before you begin clinical work
Mandatory training	Safeguarding, infection prevention, moving and handling, basic life support, fire safety, information governance, equality and diversity and more
IT setup	Logins for the electronic record system, email, rostering and e-learning platforms
Uniform	Fitting and issue, plus the dress code and what the colours signify in that organisation
Policies and procedures	Where to find them and which ones you must read before practising
Introduction to the team	Who is who, who to go to, and how the team is structured
Supervised practice	A period working alongside an experienced colleague before taking your own caseload
Preceptorship or structured support	A structured start for newly registered nurses — see below
Learning local systems	Documentation, medication management, escalation, handover, ordering and referrals
OSCE preparation	Where you arrive before sitting the OSCE, a dedicated preparation programme is often provided

Preceptorship

The NMC describes preceptorship as the structured start for newly registered nurses, midwives and nursing associates — welcoming you into the team, helping you translate knowledge into practice, building confidence, and showing how The Code applies day to day. The NMC's principles for preceptorship explicitly cover those coming to work in the UK from outside the UK. Preceptorship is not a sign that anyone doubts your ability. It is the recognised way a registered professional is settled into a new professional context, and it is worth asking a prospective employer what theirs looks like.

ONBOARDING VARIES — A GREAT DEAL

A large NHS trust with an international recruitment programme, a single independent care home and a community provider will each do this differently, at different speeds, with different levels of support. Some employers arrange airport collection, temporary accommodation and help registering with a GP and a dentist. Others do far less. This is a fair thing to ask about before you accept a role, and the answer tells you a great deal about what the first months will feel like.

Your first month in a UK healthcare setting

You already know how to nurse. What you are learning in your first month is how nursing is *organised* here — the systems, the language, the routes and the expectations of your particular clinical setting.

What you will be learning

- **Local terminology** — job titles, drug names, abbreviations, the names of forms and scores
- **Documentation systems** — which electronic or paper record, what must be recorded and when
- **Medication processes** — prescribing, checking, storage, controlled drugs, error reporting
- **Escalation procedures** — who you call, at what threshold, through which route, out of hours
- **Safeguarding processes** — how a concern is raised, to whom, and what happens next
- **Infection prevention procedures** — local policy, audit and what is expected routinely
- **Local policies** — where they live and which ones govern your practice
- **Communication styles** — handover formats, how questions are asked, how disagreement is expressed
- **Team structures** — who does what, and who is accountable for what
- **Working hours and shift patterns** — rostering, breaks, requesting leave, swapping shifts
- **How to access support** — your line manager, preceptor, practice educator, staff networks, occupational health
- **The everyday things** — travel, food, housing, banking, registering with a GP

Things that may feel different

Not better or worse — different

AREA	WHAT INTERNATIONALLY EDUCATED NURSES OFTEN NOTICE
Documentation	The volume and the detail expected can be greater than you are used to, with a strong emphasis on recording your assessment and reasoning, not only the task. "Not recorded" is generally treated as "not done".
Communication	Communication with patients tends to be direct and explanatory, with a lot of weight on explaining, obtaining consent and checking understanding. Patients and families may ask detailed questions and expect full answers.
Professional accountability	You are personally accountable for your own practice to the NMC, independently of your employer. That includes working within the limits of your competence and saying when something is outside them.
Team structures	Multidisciplinary working is usually flatter than in some systems. Nurses are expected to question, challenge and contribute to decisions — and doing so is welcomed rather than resented.
Policies	There is a written policy for a great many things, and you are expected to know where to find it and to follow it.
Clinical terminology	Drug names, equipment names, scoring systems, abbreviations and everyday clinical shorthand may all differ from what you learned.
Delegation	You may be delegating to healthcare assistants or care staff, and you remain accountable for the appropriateness of that delegation.

What is genuinely expected of you

You are not expected to know every local system on your first day. You are expected to ask questions, work within your competence and follow your employer's policies and professional standards. Every nurse who has ever moved countries has spent a first month feeling less capable than they are. It passes, and asking is the fastest way through it — not the slowest.

The mistakes that cost nurses the most time

None of these are unusual, and none of them mean somebody is a poor nurse. They are process mistakes, and every one of them is avoidable.

01 Not understanding the full process before starting

WHAT GOES WRONG

Steps are taken out of order, or a requirement appears that could have been prepared for months earlier.

WHY IT DELAYS

You wait for documents and results you could already have had.

INSTEAD

Map the whole journey first. Chapter 01 exists for exactly this.

02 Relying on outdated information

WHAT GOES WRONG

A requirement, score, fee or rule has changed since the video, blog post or WhatsApp message was made.

WHY IT DELAYS

You prepare for a requirement that no longer exists, or miss one that now does.

INSTEAD

Check every regulatory requirement on the NMC or GOV.UK website itself, on the day you need it.

03 Confusing NMC requirements with immigration requirements

WHAT GOES WRONG

Two organisations' rules are treated as one set, so one is met and the other is missed.

WHY IT DELAYS

Problems surface at the end, when they are most expensive to fix.

INSTEAD

For every requirement, ask which organisation set it, then check with that organisation.

04 Misunderstanding the English requirements

WHAT GOES WRONG

The wrong test or version is booked, sub-scores are missed, results expire, or the combining rules are misread.

WHY IT DELAYS

Another test, another fee, another wait for a date — often several months.

INSTEAD

Read the current NMC guidance, including the combining rules, before you book anything.

05 Waiting too long to collect documents

WHAT GOES WRONG

Documents are only requested when an organisation asks for them.

WHY IT DELAYS

Police certificates, transcripts and translations are outside your control and can take months.

INSTEAD

Start now, before any application is open. Use the tracker in chapter 10.

06 Assuming NMC registration automatically means a job

WHAT GOES WRONG

Registration is completed and nothing has been done about finding an employer.

WHY IT DELAYS

The job search starts from zero, with deadlines now running.

INSTEAD

Research employers and roles in parallel with your registration.

07 Assuming a job offer automatically means a visa

WHAT GOES WRONG

Plans, resignations and flights are arranged before any immigration decision.

WHY IT DELAYS

A visa is decided by the Home Office on its own evidence, and it may take longer than expected.

INSTEAD

Treat the visa as a separate stage with its own requirements and its own timeline.

08 Not researching the employer

WHAT GOES WRONG

A role is accepted without knowing the setting, the support, the location or the shift pattern.

WHY IT DELAYS

Poor matches lead to unhappy first months and early moves that cost time and money.

INSTEAD

Read the job description, the person specification and the published inspection reports.

09 Preparing for the CBT too late

WHAT GOES WRONG

A date is booked first and revision begins afterwards.

WHY IT DELAYS

Attempts are limited, resits have waiting periods, and using all three closes your application for six months.

INSTEAD

Prepare until your practice results are consistently strong, then book.

10 Preparing for the OSCE too late

WHAT GOES WRONG

Preparation begins after arrival in the UK, alongside a new job, a new country and jet lag.

WHY IT DELAYS

Resits cost money and time, must be at the same centre, and a third failure means six months' wait.

INSTEAD

Start while still in India using the NMC's published materials, and use any employer preparation on top of that.

11 Memorising rather than understanding

WHAT GOES WRONG

Scripts and answer patterns are learned by heart for assessments and interviews.

WHY IT DELAYS

Stations and questions change, and memorised answers collapse under any follow-up.

INSTEAD

Understand why each step exists. Then you can adapt to whatever you are actually asked.

12 Applying for roles without reading the job description

WHAT GOES WRONG

Applications go to unsuitable roles, or to roles the applicant is not yet eligible for.

WHY IT DELAYS

Rejections consume weeks and teach you nothing.

INSTEAD

Read the person specification and answer it point by point. Fewer, better applications.

13 Not preparing for competency-based interviews

WHAT GOES WRONG

A clinically excellent nurse arrives with no prepared examples and no structure.

WHY IT DELAYS

Strong candidates are rejected for reasons that have nothing to do with their nursing.

INSTEAD

Prepare six to eight real examples and practise them in the STAR structure.

14 Not keeping copies of documents

WHAT GOES WRONG

A document submitted once cannot be produced again when a different organisation asks for it.

WHY IT DELAYS

Replacing overseas documents from the UK can take months.

INSTEAD

Scan everything, store it securely in the cloud, and keep the originals together and safe.

15 Depending entirely on someone else to manage the process

WHAT GOES WRONG

Nobody knows where the application stands, and deadlines pass unnoticed.

WHY IT DELAYS

The applications are yours. You are accountable for the evidence in them, whoever helps you.

INSTEAD

Accept support gladly, but keep your own account access, your own records and your own timeline.

16 Trusting a summary instead of the source

WHAT GOES WRONG

A decision is made on the strength of a summary — a forum post, a video, a well-meaning friend, or a guide like this one — without ever opening the page it summarises.

WHY IT DELAYS

Everything above, essentially. This one sits behind a good many of the others.

INSTEAD

Two bookmarks: the NMC and GOV.UK. Check there first, every time.

Your complete UK nurse checklist

Everything in one place. Print it, keep it, work through it. Not every line will apply to you, and the order will vary — but nothing here is optional to *think* about.

Before you start

☐ Understand the UK nursing system

☐ Check your eligibility

☐ Research the NMC process on the NMC's own site

☐ Research the healthcare settings open to you

☐ Decide your field of registration

☐ Plan your time and your budget

Documents

☐ Passport — valid, and valid well ahead

☐ Qualification documents

☐ Academic transcripts

☐ Registration documents

☐ Evidence of any name change

☐ Employment evidence, with no unexplained gaps

☐ References, with permission and correct contacts

☐ English language evidence

☐ Police clearance from each relevant country

☐ Health declaration

☐ Certified translations where needed

☐ Secure digital copies of everything

NMC

☐ Complete the route assessment

☐ Create your MyNMC account

☐ Submit your eligibility and qualification application

☐ Pay the qualification evaluation fee

☐ Complete qualification and registration verification

☐ Meet the English language requirements

☐ Prepare for and pass the CBT

☐ Prepare for and pass the OSCE

☐ Complete the face-to-face identity check

☐ Submit health, character and indemnity evidence

☐ Pay the registration fee

☐ Obtain NMC registration and your PIN

Job search

☐ Prepare your supporting statement, and a CV where one is asked for

☐ Research employers across all four nations

☐ Identify suitable roles and settings

☐ Read every job description and person specification

☐ Apply

☐ Prepare six to eight STAR examples

☐ Prepare for interviews

☐ Receive and review your offer in writing

Sponsorship and immigration

☐ Confirm the employer's full legal identity

☐ Check the register of licensed sponsors

☐ Confirm the sponsorship arrangements for your role

☐ Receive your Certificate of Sponsorship where applicable

☐ Check its details against your job offer

☐ Identify the correct visa route on GOV.UK

☐ Gather the evidence GOV.UK lists for that route

☐ Complete the visa application

☐ Complete identity and biometric requirements

☐ Receive the immigration decision

☐ Set up and secure your UKVI account access

Relocation

☐ Accommodation confirmed, with address and access

☐ Travel booked, with a plan for delays

☐ Banking arrangements for your first weeks

☐ Phone and SIM, with your old number kept active

☐ Original documents packed in hand luggage

☐ Digital backups accessible from anywhere

☐ Employer contact details, including out of hours

☐ Journey to your place of work understood

☐ Emergency contacts shared with family

☐ Health and medication information carried with you

☐ Clothing for the UK climate

☐ Family and financial arrangements at home settled

Starting work

☐ Employer induction

☐ Identity and right-to-work checks

☐ Occupational health where applicable

☐ Mandatory training

☐ IT access and systems training

☐ Uniform and dress code

☐ Local policies read

☐ Supervised or supported practice period

☐ Preceptorship or structured support arranged

☐ Local systems understood — documentation, medication, escalation

☐ Know how to access support

☐ Start your role

Keep going after you start

Registration is not the end of the professional process. You will pay an annual retention fee to stay on the register, and you will revalidate periodically, which involves recording practice hours, continuing professional development, feedback and reflection. Start keeping that record from your first week — it is far easier than reconstructing three years of it later. The NMC publishes what revalidation requires.

ONE LAST WORD ON THE CHECKLIST

If this looks like a lot, that is because it is a lot — but it is a lot spread across many months, and almost none of it is difficult in itself. The nurses who find this process manageable are not the cleverest or the most experienced. They are the ones who knew what was coming, kept their own records, and started the slow items early.

Ready to explore your UK nursing journey?

You don't need to have everything figured out today. The UK nursing process is made up of several stages, and understanding the journey is the first step.

You have now seen the whole route — who decides what, what you need to prepare, where the delays come from and what your first months might look like. That is more than most nurses know when they begin. The next step is simply to decide where you are on it.

About Northward Care

Northward Care helps Indian nurses stay connected to UK nursing opportunities, practical resources and relevant career information. We publish what we know about the route in plain language, point you to the official source for anything that changes, and get in touch if a live role looks like a match for your profile.

YOUR NEXT STEP

Join the Northward Care Nurse Register

Register your details so we can keep you connected to relevant UK opportunities if and when they arise.

Join the Register

Joining the register simply allows Northward Care to contact you about relevant opportunities and resources. It does not create any obligation on your part, and you can ask us to remove your details at any time.

What joining does — and what it does not

Joining the Register is a way of staying connected. It is not a job application, and it is not a guarantee of anything. Northward Care cannot promise you a job, sponsorship, visa approval, NMC registration or a placement — those decisions are made by employers, by the Home Office and by the NMC, each against their own requirements. What we can do is let you know when we have an opportunity or a resource that looks relevant to you, and point you to the official source for anything that matters.

Northward Care publishes general information about NMC registration and UK immigration rules. It is not immigration advice and must not be relied on as advice about your own circumstances. For advice on your own position, speak to an adviser regulated by the Immigration Advice Authority.

Official sources & useful links

These are the organisations whose answers actually count. Everything in this guide was checked against them in September 2026. Requirements change — check these before you act.

Nursing and Midwifery Council (NMC) — professional registration

NMC home

<https://www.nmc.org.uk/>

Information for internationally trained applicants

<https://www.nmc.org.uk/registration/information-for-internationally-trained-applicants/>

Trained outside the UK — the step-by-step how-to guide

<https://www.nmc.org.uk/registration/joining-the-register/register-nurse-midwife/trained-outside-uk/how-to-guide/>

Costs and timescales

<https://www.nmc.org.uk/registration/joining-the-register/register-nurse-midwife/trained-outside-uk/how-to-guide/check-ready/costs/>

English language requirements

<https://www.nmc.org.uk/registration/joining-the-register/english-language-requirements/>

Accepted English language tests and the test-combining calculator

<https://www.nmc.org.uk/registration/joining-the-register/english-language-requirements/accepted-english-language-tests/>

Test of Competence — overview

<https://www.nmc.org.uk/registration/joining-the-register/toc/>

Test of Competence for nurses and midwives

<https://www.nmc.org.uk/registration/joining-the-register/toc/toc-nursing-and-midwifery/>

The CBT

<https://www.nmc.org.uk/registration/joining-the-register/toc/toc-nursing-and-midwifery/cbt/>

The OSCE

<https://www.nmc.org.uk/registration/joining-the-register/toc/toc-nursing-and-midwifery/osce/>

Test of Competence preparation materials, blueprints and marking criteria

<https://www.nmc.org.uk/registration/joining-the-register/toc/toc-nursing-and-midwifery/resources/>

Identity check process and requirements

<https://www.nmc.org.uk/registration/joining-the-register/id-check-process-and-requirements/>

The Code — professional standards for nurses and midwives

<https://www.nmc.org.uk/standards/code/>

Standards of proficiency for registered nurses

<https://www.nmc.org.uk/standards/standards-for-nurses/standards-of-proficiency-for-registered-nurses/>

Principles for preceptorship

<https://www.nmc.org.uk/standards/guidance/preceptorship/>

GOV.UK — immigration, sponsorship and ethical recruitment

Skilled Worker visa

<https://www.gov.uk/skilled-worker-visa>

Health and Care Worker visa

<https://www.gov.uk/health-care-worker-visa>

Knowledge of English for the Skilled Worker route

<https://www.gov.uk/skilled-worker-visa/knowledge-of-english>

Documents you must provide

<https://www.gov.uk/skilled-worker-visa/documents-you-must-provide>

Eligible occupations and codes

<https://www.gov.uk/government/publications/skilled-worker-visa-eligible-occupations/skilled-worker-visa-eligible-occupations-and-codes>

Register of licensed sponsors: workers

<https://www.gov.uk/government/publications/register-of-licensed-sponsors-workers>

Certificates of sponsorship — how they work

<https://www.gov.uk/uk-visa-sponsorship-employers/certificates-of-sponsorship>

eVisas — your digital immigration status

<https://www.gov.uk/evisa>

TB testing for visa applicants

<https://www.gov.uk/tb-test-visa>

Code of Practice for the international recruitment of health and social care personnel

<https://www.gov.uk/government/publications/code-of-practice-for-the-international-recruitment-of-health-and-social-care-personnel>

Applying for health and social care jobs in the UK from abroad

<https://www.gov.uk/government/publications/applying-for-health-and-social-care-jobs-in-the-uk-from-abroad>

Finding a role

The job sites below carry NHS and HSC vacancies. They are not where most private hospital, nursing home, hospice or agency roles are advertised — those appear on the employers' own career pages, on general job boards, or through professional networks and organisations like Northward Care. Searching only the NHS sites will show you a fraction of the registered nursing roles actually available.

NHS Jobs — England and Wales

<https://www.jobs.nhs.uk/>

NHS Scotland recruitment

<https://apply.jobs.scot.nhs.uk/>

HSC Jobs — Northern Ireland

<https://jobs.hscni.net/>

NHS Employers — recruitment of overseas nurses and midwives

<https://www.nhsemployers.org/articles/recruitment-overseas-nurses-and-midwives>

NHS Employers — Skilled Worker: Health and Care visa

<https://www.nhsemployers.org/articles/skilled-worker-health-and-care-visa>

NHS Employers — Agenda for Change pay scales

<https://www.nhsemployers.org/articles/pay-scales-202627>

Understanding settings and employers

Care Quality Commission — inspection reports, England

<https://www.cqc.org.uk/>

Care Inspectorate — Scotland

<https://www.careinspectorate.scot/>

Care Inspectorate Wales

<https://www.careinspectorate.wales/>

RQIA (Regulation and Quality Improvement Authority) — Northern Ireland

<https://www.rqia.org.uk/>

Skills for Care — registered nurses in adult social care

<https://www.skillsforcare.org.uk/Regulated-professions/Nursing/Registered-nurses/Registered-nurses.aspx>

Royal College of Nursing — nursing careers resource

<https://www.rcn.org.uk/Professional-Development/Nursing-careers-resource>

A closing note on accuracy

This guide was researched and checked against the sources above in September 2026. Registration requirements, English language requirements, test formats, test centres, fees, immigration rules, salary thresholds and eligible occupations all change — sometimes at short notice. Nothing here is advice about your own circumstances, and no part of it should be relied on in place of the current guidance published by the NMC and the UK Government. When the two disagree, they are right and we are out of date.

Questions, feedback or a correction?

If you have a question about anything in this guide, think something could be explained better, or spot something that has changed, please get in touch at hello@northwardcare.com. We would rather hear from you than have you working from something out of date — and reader questions are how this guide gets better.



Northward Care

**Understand the route
before you take it.**

northwardcare.com

hello@northwardcare.com